CASE PRESENTATION

Ananya Girish
Final Year Student
M.V.J.MC&RH

- Name: Mrs. D
- Age: 27 years
- Address: Hoskote
- Occupation: Home maker
- Educational status: 10th Std
- Husbands name: Mr. M Age: 33 yrs. Occupation: Shopkeeper
- Socio-economic status: IV (according to modified BG Prasad classification)
- LMP: 10/8/2019 EDD: 17/5/2020
- Obstetric Score: primi gravida
- Gestational age: 30 weeks 2 days
- Date of admission: 9/3/2020
- Date of examination: 9/3/2020

Patient gives H/o 7 ½ months of amenorrhea
Appreciating Fetal Movements well.

Chief Complaints: Headache since 1 day
Increased BP recording since 1 day

History of Presenting Illness:

A primi gravida with 7 ½ months of amennorhea came with complaints of headache since 1 day for which the patient consulted a local doctor and was found to have increased BP recording.

Headache is localized to forehead and occipital region. It was not associated with vomiting.

No h/o vomiting, epigastric pain, blurring of vision.

No h/o pain abdomen,leaking or bleeding PV.

History Of Present Pregnancy

- LMP:10/8/19
- Married life 1 yr 4 months
- Non-consanguineous marriage
- Blood Group: O+ve

1st Trimester

- Conceived spontaneously
- Confirmed after 1st missed period.
- UPT- was positive; confirmed by scan
- Dating scan done; showing GA corresponding to POG
- NT,NB scan done
- Folic acid tablets were taken
- No h/o burning micturition
- No h/o fever with rashes
- No h/o excessive vomiting
- No h/o easy fatigability
- No h/o spotting or bleeding per vagina
- No h/o pain abdomen
- No h/o exposure to radiation and drug intake

2nd trimester:

- Quickening at 5th month and continued to perceive fetal movements well
- Anomaly scan was done was found to be normal
- Both doses of inj. T.T. taken
- Iron and calcium tablets taken
- BP, blood Sugar, Thyroid test and other routine investigations were done and found to be normal.
- No h/o pedal edema, blurring of vision, headache.
- No h/o nausea and vomiting
- No h/o of pain abdomen, leaking or bleeding per vagina

3rd trimester:

- Fetal movement well perceived
- Continuing to take Iron and Calcium tablets
- Came with complaints of headache and raised BP readings since 1 day.
- No c/o abdominal pain
- No h/o bleed or leak per vagina

Past obstetric history

- Primi gravida

Menstrual history

- LMP: 10/8/2019
- Age of attainment of menarche – 13 years
- Regular cycle of 28 to 30 days, flow for 4-5 days
- Changes 2 pads per day
- Not associated with dysmenorrhea,
- No h/o of passage of clots

Past history:

- Not a known case of Hypertension/Diabetes/Tuberculosis/ Asthma/ Epilepsy/Thyroid disease
- No h/o blood transfusion in the past
- No h/o any recent surgery

Family history:

- No h/o Hypertension, Diabetes mellitus, asthma, TB, epilepsy
- No h/o twin pregnancy
- No h/o genetical abnormalities

Personal history:

- Mixed-diet; adequate in proteins & calories
- Sleep-Undisturbed
- Appetite-Normal
- Bowel & bladder habits-Regular

General physical examination

- Patient is conscious, cooperative well oriented to time, place, and person

Vitals

- Patient is afebrile
- Pulse - 94/min regular rhythm, good volume, normal character, vessel wall is not palpable and all peripheral pulse are felt.
- BP - 146/96 mm hg right arm, sitting position.
- Respiration – 16 cycles/minute
- No pallor, icterus, cyanosis, clubbing, lymphadenopathy, Presence of bilateral pedal edema grade 3
- Height-152cm
- Weight-62kg
- BMI-26.46kg/m^2
- Clinically Euthyroid
- Bilateral breast-Normal
- Spine-Normal

SYSTEMIC EXAMINATION:

1. CVS: S_1 S_2 normal; no murmur.
2. RS: Clear ; Normal Vesicular Breath Sounds ; B/L equal air entry.
3. CNS: No Focal Neurological Deficit.

PER ABDOMEN EXAMINATION:

- INSPECTION:
- Abdomen uniformly distended
- Linea nigrae + ; Striae gravidarum +
- No scar/ sinuses
- No dilated veins
- PALPATION:
- No local rise of temperature or tenderness
- SFH-26 cm; EFW-2.1Kg;Uterus- corresponds to 28 weeks of gestation; relaxed
- Liquor- Clinically appears adequate
- Fundal grip- broad, soft, irregular structure felt; suggestive of breech.

- Lateral grip → Right -irregular knobs like structure suggestive of limbs

  Left-Smooth, Curved-Back of the fetus suggestive of spine
- Pelvic grip- hard, globular ballotable structure felt suggestive of head, mobile

- AUSCULTATION: FHR- 136 bpm, Left spinoumblical line.

Provisional Diagnosis

Primi Gravida,30 weeks period of gestation with single live pregnancy longitudinal lie in cephalic presentation with pre eclampsia.